

9 June 2026

RELIANCE GUIDELINE

This guideline is intended to provide recommendations to applicants wishing to submit new registration applications, as well as variations, for reliance review-based evaluation. It represents the South African Health Products Regulatory Authority's (SAHPRA's) current thinking on the safety, efficacy, and quality of medicines. It is not intended as an exclusive approach. SAHPRA reserves the right to request any additional information to establish the safety, efficacy, and quality of a medicine in keeping with the knowledge current at the time of evaluation. Alternative approaches may be used, but these should be scientifically and technically justified. The Authority is committed to ensuring that all registered medicines will be of the required safety, efficacy, and quality. It is important that applicants adhere to the administrative requirements to avoid delays in the processing and evaluation of applications.

Document History

Final Version	Reason for Amendment	Effective Date [Month yyyy]
1	First publication released for implementation	31 July 2019
2	Consolidation of already published information about SAHPRA Reliance approaches	31 October 2021
3	Inclusion of references to forms and removal of reference to the list of GMP Recognised Regulators, as this information is already included in 4.01 SA Guide to Good Manufacturing Practice	31 March 2022
4	- Transfer of content to the latest SAHPRA Guideline template - Old document number 5.08 Reliance Guideline changed to SAHPGL- BAU-01 in alignment with the latest SAHPRA document coding structure - Updated to reflect new requirements for reliance submissions	May 2024
5	- Updated to align with current processes, - Added AMA and EU-M4all, and - Included the clause for MOUs SAHPRA holds under Section 4.1 below.	July 2025
6	- Added an abbreviation for Collaborative Registration Procedure (CRP). - Updated to reflect the evaluation timelines for applications reviewed through the reliance and collaborative review pathways.	December 2025
7	- Administrative and editorial changes include updates to abbreviations and sentence re- wording [see section 1 introduction, section 3, section 4.2, Section 4.3 and section 4.4 (Note)]; - In section 3 deleted of Communication to Industry and replaced with	June 2026

	Multiple Application Guideline; - Section 4.1, updated reliance pathway requirements regarding maintenance of reliance status, restriction on switching RRAs post-registration, and prior RRA approval of variations; - Section 4.3 added Screening Checklist for New Medicine application for registration (GLF-HPA-07C) and document number for validation variation template (GLF-HPA-03A) and these documents are available on the SAHPRA website; - Section 4.3 added Choosing the intended pathways enables SAHPRA to conduct technical screening more efficiently; - Section 4.5 EMA with SAHPRA and added: Sameness must be maintained from the time of product registration except for regional specific differences agreed upon at the time of registration; - Section 9 added AMA; - Section 9.1 updated and added the section on requirements applicable to finished products submitted under SAHPRA's GMP reliance pathway and Requirements applicable to active pharmaceutical Ingredients; - In section 12, added the section on requirements for routine independent lot release testing to include risk-based reliance considerations and recognition of lot release certificates from aligned NRAs and recognized WHO laboratories; and - Addition of SAHPRA GMP Guideline (SAHPGL-INSP-02) reference in section 13.	
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Glossary

Abbreviation/ Term	Meaning
AMA	African Medicines Agency
API	Active Pharmaceutical Ingredient
CHMP	Committee for Medicinal Products for Human Use
CPQ	Confirmation of WHO API Prequalification
CRP	Collaborative Registration Procedure
CTD	Common Technical Document
EA	Extension Application
EMA CVMP	European Medicines Agency Committee for Veterinary Medicinal Products
EU CP	EU Centralised Procedure
EU DCP	EU Decentralised Procedure
EU-M4all	EU-Medicines for all
EU	European Union
QIS	Quality Information Summary
QOS	Quality Overall Summary
GCP	Good Clinical Practice
GMP	Good Manufacturing Practice
GRP	Good Regulatory Practice
HCR	Holder of Certificate of Registration
MAH	Market Authorisation Holder: Equivalent to HCR: Holder of the Certificate of Registration
MoU	Memorandum of Understanding
MHLW	Ministry of Health, Labour and Welfare (Japan)
MHRA	Medicines and Healthcare products Regulatory Agency (UK)
NCE	New Chemical Entity
PEM	Pharmaceutical Evaluation Management
Package Leaflet	Equivalent to PIL
PAR	Public assessment reports
PBRER	Periodic Benefit-Risk Evaluation Report

PI	Professional information
PIC/S	Pharmaceutical Inspection Cooperation Scheme
PIL	Patient Information Leaflet
PMDA	Pharmaceutical and Medical Devices Agency
PQ	Pre-qualification
PSUR	Periodic Safety Update Report
Q and BE	Quality and Bioequivalence
SAHPRA	South African Health Products Regulatory Authority
Sameness	Sameness refers to two products having identical essential characteristics. It is an important aspect of reliance that ensures that the same product that was assessed by the reference regulatory authority is the same one being applied for to the relying authority. The essential characteristics that are required to be the same or sufficiently similar include but are not limited to: manufacturing sites and/ or suppliers of the API, FPP, and excipients (/IPs); manufacturing processes and control of both the API, FPP, and excipients; pharmaceutical form, strength, use, qualitative and quantitative composition. Additionally, the results of supporting studies of safety, efficacy, and quality, indications, and conditions of use should be the same.
SmPC	Summary of Product Characteristics: Equivalent to PI
SRA	Stringent regulatory authority
SwissMedic	Swiss Agency for Therapeutic Products
Well-established use	Medicines that include an active ingredient that has results supported by scientific literature.
Recognition	A streamlined registration/ approval process based on directly recognising the outcome of a review from an RRA with which SAHPRA shares a recognition agreement.
RMP	Risk Management Plan
RRA	Recognised Regulatory Authority – a term used to refer to the list of regulatory authorities with which SAHPRA aligns itself.
TGA	Therapeutic Goods Administration (Australia)
UK	United Kingdom
US FDA	United States Food and Drug Administration
WHO	World Health Organization
WHO-(WLA)	WHO listed Authorities (WLA)
ZAZIBONA	ZAZIBONA is a collaborative medicines registration initiative in Southern Africa focusing on dossier assessments and good manufacturing practice (cGMP) inspections.

APVMA	Australian Pesticide and Veterinary Medicinal Products
JMAFF	Japanese Ministry of Agriculture, Forestry, and Fisheries
IPI	Inactive Pharmaceutical Ingredient
MAGHP	Marketing Authorisation for Global Health Products
HC	Health Canada
MOU	Memorandum of Understanding
AMRP	Abbreviated Medicine Review Process
UKVMD	United Kingdom Veterinary Medicines Directorate
MEDSAFE	New Zealand Medicines and Medical Devices Safety Authority
FPP	Finished Pharmaceutical Product

1. INTRODUCTION

The South African Health Products Regulatory Authority (SAHPRA) has decided to harmonise certain SAHPRA medicine policies and procedures with those of the European Medicines Agency (EMA). By doing so, SAHPRA will reflect global best practice in terms of the safety, quality and efficacy of health product regulation. In line with this, SAHPRA has adopted the reliance review approach as a pathway for the registration of medicines. This approach allows the Authority to leverage evaluation efforts done by a recognised regulatory authority (RRA) with which it aligns, making its evaluation process more efficient and enhance market access.

Please note: Unless mentioned otherwise, where EMA guidelines adopted in South Africa include references to European Union (EU) legislation, the requirements contained in the referenced EU legislation are not applicable to the evaluation of medicines by SAHPRA. South African legislation will apply wherever relevant and current.

1.1 Purpose

This guideline is intended to provide information and guidance to applicants/ Holders of Certificate of Registration (HCRs) on the prescribed requirements and process to be followed, in cases where a new registration or variation application is submitted to SAHPRA with the applicant/HCR requesting a reliance-based evaluation.

1.2 Scope

This guideline applies to new registration applications for human medicines, including biologicals. It is also applicable to veterinary medicines and variation applications.

2. LEGAL PROVISION

The Medicines and Related Substances Act (101/1965), as amended, details under Section 2B(2)(a)(2)(b) that:

(2)(a) The Authority may liaise with any other regulatory authority or institution and may, without limiting the generality of this power, require the necessary information from, exchange information with, and receive information from any such authority or institution in respect of –

- (i) matters of common interest; or*
- (ii) a specification investigation; and*

(2)(b) enter into agreements to co-operate with any regulatory authority to achieve the objects of this Act.

Regulation 16 to this Act, furthermore, states that:

(8) In the case where a medicine in respect of which an application for registration is made is or was registered with any regulatory body outside the Republic, the following information in respect of such medicines shall accompany the application:

- (a) a copy of the certificate of registration;*
- (b) professional information relating to the medicine;*
- (c) conditions of such registration; and*
- (d) any other information as may be required by the Authority.*

3. RELIANCE-BASED EVALUATION PATHWAYS

The World Health Organization (WHO) defines reliance as the act whereby the regulatory authority in one jurisdiction may consider and give significant weight to – i.e., totally or partially rely upon – evaluations performed by another regulatory authority or trusted institution in reaching its own decision. The relying authority remains responsible and accountable for decisions taken, even when it relies on the decisions and information of others. Wherever possible, SAHPRA will leverage these pathways, relying on the evaluation efforts of Recognised Regulatory Authorities (RRAs) to reduce evaluation times.

Reliance-based evaluation pathways of medicine applications for new registrations and variations in South Africa will follow one of three evaluation/ review pathways:

- a) Abridged review
- b) Verified review
- c) Recognition

NOTE: Pathways (a), (b), and (c) replace the prior Abbreviated Medicines Review Process (AMRP). The application follows different reliance-based evaluation pathways between Clinical, Quality and Bioequivalence (see Section 4 below and refer also to the **Multiple Application Guideline (SAHPGL-HPA-08)** on Clones and Replicas).

4. GENERAL DESCRIPTIONS OF THE RELIANCE-BASED EVALUATION PATHWAYS

- a) **Abridged review:** A streamlined review based primarily on full assessment reports from RRAs, replacing the need to evaluate all data (and summaries thereof) submitted in support of an application.
- b) **Verified review:** A streamlined review based primarily on verifying, instead of evaluating,

information submitted in the application against information that has already been approved by SAHPRA or an RRA. Note that full assessment reports are required for Quality and Bioequivalence PEM-verified reviews as a fallback option for evaluators.

- c) **Recognition:** A streamlined registration/ approval process based on directly recognising the outcome of a review from an RRA with which SAHPRA shares a recognition agreement.

NOTE: SAHPRA is currently in the process of negotiating recognition agreements with RRAs. Once such an agreement is in place, SAHPRA will publish a framework for the practical implementation thereof. The guiding principle is that applications approved by RRAs with which SAHPRA shares a recognition agreement may not need to be evaluated separately by SAHPRA. Please note that this is not to be confused with collaborative/ work-sharing procedures (e.g., ZAZIBONA).

4.1 SAHPRA's Recognised Regulatory Authorities

To qualify for a reliance evaluation pathway, an application must have been approved by one or more of the RRAs with which SAHPRA aligns itself.

SAHPRA's current RRAs include:

- European Union Centralised Procedure (EU CP)
- European Union Decentralised Procedure (EU DCP) (no restrictions on which member state acts as the reference member state)
- Health Canada
- Medicines and Health Products Regulatory Agency (MHRA (UK))
- Ministry of Health, Labour and Welfare (MHLW)/ Pharmaceutical and Medical Devices Agency (PMDA) - Japan
- Swiss Agency for Therapeutic Products (SwissMedic)
- Therapeutic Goods Administration (TGA) -Australia
- US Food and Drug Administration (US FDA)
- WHO listed Authorities (WLA)

SAHPRA has certain MOUs and agreements in place with organisations and regulatory authorities. The scope of these agreements determines the applicability of certain reliance or work-sharing procedures. Information on the use of these agreements or related procedures is communicated by SAHPRA when applicable.

The following additional procedures can be used for reliance/ collaborative review, which are not strictly regulatory authorities:

- World Health Organization (WHO) collaborative registration process:
 - Prequalification (PQ)
 - SRA approved
- ZAZIBONA collaborative procedure
- European Medicines Agency
 - Mutual Recognition procedure and National Procedures within the EU
- SwissMedic Marketing Authorization for Global Health Products (MAGHP) procedure
- PIC/s Member States
- AMA
- EU-M4all

SAHPRA's recognised regulatory authorities for registration of veterinary medicines include:

- EMA_CVMP
- US FDA
- JMAFF
- APVMA
- UKVMD
- HC
- MEDSAFE
- SwissMedic
- AMA
- Veterinary Zazibona

Should the applicant wish to pursue reliance for product registration, the reliance status should be maintained throughout the lifecycle of the product. The Authority does not permit the switching of RRAs post registration. Should a mechanism for switching RRAs after registration develop in future, this will be communicated through subsequent updates to the guideline. Applicants are therefore required to ensure that all variations are

submitted to and approved by the relevant RRA prior to submission to the Authority.

SAHPRA Evaluation Timelines for Collaborative Review and Reliance Pathways

SAHPRA's timelines for the evaluation of applications submitted under the collaborative review pathways are as follows:

- WHO Prequalification (WHO PQ): 90 working days
- WHO SRA Collaborative Registration Procedure (WHO SRA CRP): 90 working days
- African Medicines Agency (AMA): 90 working days
- Marketing Authorisation for Global Health Products (MAGHP): 90 working days
- ZAZIBONA Collaborative Procedure: 90 working days
- EU-M4ALL (EU Medicines for All): 90 working days

The evaluation timeline for applications qualifying for the abridged and verified review pathways is 200 working days.

Note: The timelines indicated above exclude the period allocated for technical screening and the time during which the application is with the applicant for response.

4.2 Independent application of reliance for Quality & Bioequivalence (quality and bioequivalence, and clinical)

Application(s) often differ in complexity between Clinical, Quality and Bioequivalence evaluation. For example, a typical application for a generic/ multisource medicine requires a relatively straightforward verification of PIs for Clinical review, however Quality and Bioequivalence face the added complexity of bioequivalence assessments. Consequently, SAHPRA's reliance pathways are applied independently for Clinical, Quality and Bioequivalence. **This has the following two key implications:**

- Evaluation pathways may differ for Clinical, Quality and Bioequivalence evaluation (e.g., Clinical may follow a verification procedure, while Quality and Bioequivalence follow a full review, depending on the nature of the application and the quality of reliance documents submitted);
- The RRAs referenced in an application may differ for Clinical, Quality and Bioequivalence evaluation (e.g., Clinical may rely on the SAHPRA-approved local innovator PI and latest EMA SmPC as part of a verified review, while the Quality and Bioequivalence evaluation may rely on information approved by the TGA).

This approach widens the use of reliance, by not limiting an application to the same pathway/reference RRA for Clinical, Quality and Bioequivalence evaluation.

4.3 Technical Screening of Applications

Applicants must indicate the intended evaluation pathways for Clinical, Quality and Bioequivalence evaluation, along with a brief motivation. The intended evaluation pathways should be indicated on the Screening Checklist for New Medicine application for registration (GLF-HPA-07C) / the variation validation template (GLF-HPA-03A) in the relevant sections. These documents are available on the SAHPRA website. By choosing the intended pathways enables SAHPRA to conduct technical screening more efficiently and prevents unnecessary delays.

Decisions related to an application's final evaluation pathway and the extent of reliance on an RRA's evaluation are fully at SAHPRA's discretion. These decisions will depend on the availability and quality of reliance documentation submitted. SAHPRA will share screening queries to the respective applicant(s) who submitted insufficient reliance documentation, to maximise the number of applications that can qualify for abridged and verified reviews. Where applicable, applications will default to a full review in the absence of a suitable reliance pathway.

4.4 Assessment Reports

Where indicated as a requirement for an abridged or verified review, applicants are to provide SAHPRA with full assessment reports from an RRA (submitted in Module 1.10).

The following requirements apply:

- Full assessment/ evaluation reports must include, at a minimum, the safety, efficacy, and quality report(s) prepared by the RRA upon which the registration/ approval decision was based (refer to Table 2 for a list of documents required for Q-BE).
- Where full assessment/ evaluation reports from the RRA are in languages other than English, the applicant must submit translated versions in line with Regulation 16 (4).

NOTE: SAHPRA requires assessment reports to be submitted directly by the applicant. Failure to submit these reports will likely result in the application defaulting to a full review, which may significantly extend the evaluation timeline.

For applications where the USFDA is the RRA, a letter of confirmation from the USFDA marketing

authorisation holder (proprietary owner) confirming that they are the legal holder of the proprietary information and that they wish to share information about this product with SAHPRA should be provided.

For applications with RRAs listed in Section 4.1 except for WHO collaborative procedures and US FDA-based applications, SAHPRA will not source the RRA assessment reports, and the letter of access for reliance (GLF-PEM-02E) will not be applicable. Applicants are required to source the full reports from the Marketing Authorisation Holder.

NOTE: SAHPRA retains the right to request additional information from applicants about the application. If no full assessment reports are received from the RRA within three (3) months of the request, the application will be reviewed using a full review.

All other Marketing Authorisation Holders that wish to submit assessment reports directly to SAHPRA should use the dedicated email address: reliancereports@sahpra.org.za. The SAHPRA allocated application number must be specified when sending the reports.

4.5 Quality Variations

For products registered through reliance only:

Type IA

- For Type IA & IA_{IN} variations submitted to and implementable in the RRA, the applicant must provide proof of submission to the RRA, and if available, the acknowledgement or approval communication. The list of variations must be reflected in the covering letter submitted to the RRA, and these must concur with the covering letter submitted to SAHPRA. Any rejection/ query letter regarding these Type IA variations must be provided. These Type IA variations submitted to the RRA must be classified as Type IA and may be grouped as a single variation to SAHPRA.

Type IB

- Type IB variations approved by the RRA may be classified as a Type IA variation provided the applicant can provide approval/ acknowledgement communication and assessment reports from the RRA at the time of submission to SAHPRA. These variations may be grouped as a single Type IA variation. The list of variations must be reflected in the covering letter submitted to the RRA, and these must concur with the covering letter submitted to SAHPRA. Any rejection/ query letter regarding these Type IB variations must be provided.

- If full assessment reports are not available, the change will remain a Type IB variation and must be submitted under the applicable code as per the SAHPRA variations Addendum for Human and Veterinary Medicines (SAHPGL-HPA-06).

Type II

- Type II variations approved by the RRA may be classified as Type IB variations provided the applicant can provide approval/ acknowledgement communication and assessment reports from the RRA at the time of submission to SAHPRA. Each change must be classified as a separate Type IB variation.
- If full assessment reports are not available, the change will remain a Type II variation and may only be implemented once an approval letter is issued by SAHPRA.
- For classification, conditions, and required documents, refer to the Variation Addendum for Human and Veterinary Medicines (SAHPGL-HPA-06).

Please note that for changes which affect both SAHPRA Inspectorate and Quality Units, the above classifications are not applicable.

All required supporting documents and amended sections of the dossier must be submitted to SAHPRA regardless of the evaluation pathway.

Sameness must be maintained from the time of product registration except for regional specific differences agreed upon at the time of registration.

5. PRINCIPLES OF RELIANCE-BASED EVALUATION – CLINICAL

For PI/PIL content, SAHPRA will be using reliance wherever applicable. As per the documentation requirements in Section 6 this typically involves the submission of the latest approved (and attainable) PI/PIL from a regulatory authority with which SAHPRA aligns itself. SAHPRA considers PI/PILs previously approved by the EU (either Centralised Procedure or Decentralised Procedure) as a default reference for reliance pathways. Alternatively, applicants can provide an approved PI/PIL from any other RRA.

NOTE: An application for an API that has not yet been registered by SAHPRA will be considered as a New Chemical Entity (NCE) in South Africa, regardless of whether the molecule has already been registered by other regulatory authorities.

5.1 Abridged Review

The abridged review is based primarily on the overviews of pre-clinical and clinical data in CTD Modules 2.4 and 2.5. All supporting documents as stipulated in Section 6 of this guideline should be included in the submission to qualify for an abridged review.

All NCE and biological applications, generic applications with clinical data, Type II variations, and EAs that have prior approval from an RRA will be considered for an abridged review. In addition, all applications for biosimilar medicines will be considered for an abridged review.

An abridged review is indicated specifically for the following types of applications:

5.1.1 Monocomponent medicines

- For registration of an NCE already approved by an RRA
- For registration of an NCE based on well-established use (relying on literature), where the medicine has already been registered on the same basis by an RRA
- For a monocomponent multisource medicine/ generic registered by an RRA, and where clinical data generated with the generic has been supplied in support of the application
- Biological medicine registered by an RRA
- Biosimilar medicine where the reference biological medicine has already been registered by SAHPRA

5.1.2 Multicomponent medicines

- For a multicomponent fixed dose combination of two or more chemical entities, where the combination is not registered by SAHPRA, but registered by an RRA

5.1.3 Type II variations

- For Type II variations where the amendment applied for has already been approved by an RRA (e.g., additional/ amended therapeutic indications, safety amendments, posology, and method of administration)

5.1.4 EAs

- For all EAs that have not yet been approved by SAHPRA for a given molecule but have been approved by an RRA.

5.2 Verified Review

The verified review is initiated to limit the evaluation time of applications for APIs already registered by SAHPRA. The verified review is effectively a comparison of an applicant's proposed PI against an up-to-date reference PI (from a Clinical safety perspective). The primary reference is the latest approved PI of the associated local innovator product. The latest-approved foreign innovator PI may be supplied as an additional/ alternative reference only where the local innovator is materially outdated or no longer marketed (see SAHPGL-CEM-02, a Guideline on Professional Information for Human Use for which sections require complete localisation to the SA innovator product).

All Type IB variations and generic applications (without clinical data) for APIs already registered by SAHPRA will be considered for a verified review. In addition, EAs that have already been approved by SAHPRA will be considered for a verified review.

A verified review is indicated specifically for the following types of applications:

5.2.1 Monocomponent medicines

- For duplicates/ clones of medicines registered by SAHPRA
- For a multisource medicine/ generic with identical therapeutic indications, formulation/ dosage form, and strength for APIs previously approved by SAHPRA

5.2.2 Multicomponent medicines

- For a multicomponent fixed dose combination of two or more chemical entities, where the combination is already registered by SAHPRA

5.2.3 Type IB variations

- For all Type IB variations reviewed by SAHPRA

5.2.4 EAs

- For all EAs that have already been approved by SAHPRA for a given molecule
- For all EAs related to new pharmaceutical forms which follow the same route of administration as that which has already been approved by SAHPRA (e.g., EA for a capsule, where SAHPRA has already approved use of a tablet)¹

¹Regardless of whether SAHPRA or an RRA has previously approved the EA for a given molecule (i.e., the EA

for a capsule may not have been approved by SAHPRA or an RRA, but the application qualifies for verification as SAHPRA has previously approved the same [oral] route of administration).

6. DOCUMENT/ DATA REQUIREMENTS FOR NEW REGISTRATION – CLINICAL

6.1 Abridged Review Requirements

NOTE: Some requirements may not apply to a certain application type for abridged review.

6.1.1 Abridged review requirements:

- (i) Applicant cover letter (M1.0)
- (ii) Proposed PI and PIL (M1.3)
- (iii) Registration status and dates of approval with other regulatory authorities. This is stated in the ME&R section, where it is a requirement
- (iv) Applicants are requested to highlight SAHPRA's RRAs on this list
- (v) Risk Management Plan (RMP), if applicable (M1.13)
- (vi) Latest Periodic Safety Update Report (PSUR)/ Periodic Benefit-Risk Evaluation Report (PBRER) if already registered by an RRA, if applicable – (M5)

Preclinical data (proof of concept, in vitro/ in vivo data, animal data)

- (i) Overview of preclinical data (M2.4)
- (ii) Synopsis of preclinical findings of relevance to humans (M2.6)
- (iii) Preclinical data expert report from the applicant (M2.4)
- (iv) Full preclinical data (M4)

Clinical data

- (i) Overview of clinical data (incl. safety, efficacy, pharmacology, and benefit/risk analysis) (M2.5)
- (ii) Clinical expert reports on safety and efficacy from the applicant (M2.5)
- (iii) Synopsis of each clinical study included in the application (M2.7)
- (iv) Full clinical study data with formulation as applied for (FAAF) (M5)
- (v) Studies demonstrating pharmacology, including mechanism of action and pharmacotoxicology (M5)
- (vi) Studies demonstrating pharmacodynamic properties (M5)

- (vii) Studies demonstrating pharmacokinetic properties, including pharmacodynamic/ pharmacokinetic (PD/PK relationship, and where relevant, pharmacokinetic properties in special populations (e.g., hepatic, renal, gender, race, elderly, children, other age groups) and PD/PK interactions with other medicines relevant to the indication and target population (M5)
- 6.1.2 Rapporteur assessment reports from RRAs, if available (M1.10)
- 6.1.3 The relevant reference PI approved by an RRA (M1.10.3)
- 6.1.4 Correspondence between the Applicant and other reference RRAs concerning queries relating to safety, efficacy, risk/ benefit, and RMP issues (if not included in the assessment report). Detailed explanation/ reasons if registration/ approval was refused by a Regulator with which SAHPRA aligns itself (M1.10).

6.2 Verified Review Requirements

NOTE: Some requirements may not apply to a certain application type for verified review.

- 6.2.1 Verified review requirements (i – v) (refer to 6.1.1 above)
- 6.2.2 Verified review requirement (vi) if/ when applicable for specified molecules and indications (refer to 6.1.1 above)
- 6.2.3 The relevant primary reference innovator PI approved by SAHPRA (M1.3)
- 6.2.4 The relevant secondary reference PI approved by an RRA, if applicable in instances where the local innovator PI is materially outdated (M1.3)

7. PRINCIPLES OF RELIANCE-BASED EVALUATION - QUALITY & BIOEQUIVALENCE

Reliance-based evaluation will be based on the following principles:

- Reliance is applicable for both new registration and variation applications (Type IB and Type II).
- The application submitted for registration by SAHPRA should be the same as the most updated product on record at the RRA, i.e., all **approved** variations for the RRA's registered product should be incorporated in the application submitted for registration by SAHPRA. Pending variations with the RRA should **not** be included in the application submitted to SAHPRA for the application to qualify for reliance.

7.1 Abridged Review Requirements

An abridged review is a reliance-based review comprising:

- Validation by SAHPRA to ensure that the product application submitted for registration by SAHPRA is the same as the product registered by the specified RRA
- Evaluation of Module 1: Regional administrative information (as required)
- Evaluation of specific aspects of the dossier, depending on the type of application submitted, an **abridged review** applies to the following types of applications:
 - (i) For a new registration application for a generic medicine already registered by an RRA
 - (ii) For a new registration for a WHO PQ product:
- Applicants are required to follow SAHPRA's process for the WHO Collaborative Registration Procedure <https://www.sahpra.org.za/document/expressions-of-interest-who-pre-qualification-collaborative-registration-procedure/>
 - (i) For a Type II variation where the variation applied for has already been approved by an RRA

7.2 Verified Review Requirements

A verified review is a reliance-based review comprising:

- Validation by SAHPRA to ensure that the product application submitted for registration by SAHPRA is the same as the product registered by the specified RRA
- Evaluation of Module 1: Regional administrative information (as required). A **verified review** applies to the following types of applications:
 - (i) For a new registration application for an NCE medicine already registered by an RRA
 - (ii) For a Type IB variation where the variation applied for has already been approved by an RRA

8. DOCUMENT/ DATA REQUIREMENTS FOR NEW REGISTRATION – QUALITY/ BIO-EQUIVALENCE

To qualify for a reliance-based review, an applicant needs to submit additional documentation beyond the

documentation required for a full review (see Tablet 1 below).

Table 1: Documentation required for reliance-based evaluation.

Document required	Applicable types of applications
Completed abridged review template	5.1 i, ii
Completed verified review template	5.2 i
• Full assessment/ evaluation reports from the RRA where the product is registered, • Details of the outcomes of the application in all jurisdictions where it has been submitted, • Foreign registration certificate(s), and • SmPC, a copy of the patient information leaflet (PIL) and label of the product that has been registered by the RRA, • If available: initial scientific assessments, regulatory correspondence with the sponsor/ applicant, follow-up assessments, and any other documentation from the RRA related to the final registration decision, and • If available and where applicable: risk management plans and on-site inspection reports (or equivalent), for example GCP/ GRP. This does not include the data package filed with the RRA	i, iv i, iii
Letter of approval from the RRA	5.1 iv iii
Declaration: Sameness	5.1 i, ii i

Table 2: Documents that comprise a complete assessment of each RRA.

The table below lists the documents that comprise a complete assessment for each RRA. The full set of

documents must be submitted in your reliance report-based application.

RRA	Required documentation
Therapeutic Goods Administration, Australia	• Comprehensive details of all studies submitted and assessed • All assessment report(s) • Questions from the regulator to the Market Authorisation Holder (and answers) • Summaries of meetings with TGA (including presubmission advice, where relevant) • Approval letter • Post marketing reviews • Note: All relevant milestone dates are specified in the submission evaluation plan.
European Medicines Agency (EMA)	• Comprehensive details of all studies submitted and assessed • Centralised procedure assessment reports (where applicable): • Day 80 Quality, Non-Clinical, Clinical, and Overview Assessment Reports • Day 120 List of Questions (and answers) • Day 150 Quality, Non-Clinical, Clinical, and Overview Assessment Reports • Day 180 Joint Assessment Report • Day 180 List of Outstanding Issues • Final Assessment Report • Decentralised procedure assessment reports (where applicable): ○ All assessment reports ○ Questions from the regulator to the Market Authorisation Holder (and responses)

RRA	Required documentation
	○ Summaries of meetings with the EMA and/ or assessors (including presubmission advice, where relevant) ○ Committee for Medicinal Products for Human Use (CHMP) Summary of Opinion ○ Any other questions from the regulator to the Market Authorisation Holder ○ Letter of undertaking ○ European Commission decision ○ Risk Management Plan review(s) ○ Post marketing review(s) (e.g., Periodic Safety Update Reports)
Pharmaceutical and Medical Devices Agency (PMDA), Japan	• Comprehensive details of all studies submitted and assessed • Discussion documents, questions from PMDA and answers provided, and finalised minutes from Scientific Consultation Meetings (if applicable) • Outcome of Orphan designation, priority or SAKIGAKE determination (if relevant) • Copies of questions and answers exchanged between Sponsor and PMDA • Un-redacted English Translated Review Report consisting of: ○ Review Report 1 ○ Review Report 2 ○ Review Result ○ Report on the Deliberation Results ○ Approval Letter ○ Post-marketing review(s) (e.g., Re-

RRA	Required documentation
	examination Review Report, Periodic Safety Reports)
Health Canada	• Comprehensive details of all studies submitted and assessed • Screening: Screening Report • Clinical Review: Pharmaceutical Safety and Efficacy Assessment Report (PSEAR) • Quality: Quality Evaluation Summary (QES) and Manager's Memo • Bioequivalence: Comprehensive Summary – Bioequivalence (CS-BE) and Manager's Memo • Biostatistics: Biostatistics Consult Report (if applicable) • Risk Management Plan: Risk Management Plan Assessment Report (if applicable) • Questions from the regulator to the Market Authorisation Holder (and responses) • Summaries of meetings with Health Canada (including presubmission advice, where relevant) • Final Manager's Memo, and Executive Summary
Medicines and Healthcare products Regulatory Agency (MHRA), United Kingdom	• Comprehensive details of all studies submitted and assessed • All assessment reports as part of the iterative process • Questions from the regulator to the Market Authorisation Holder (and responses) • Committee for Medicinal Products for Human Use • (CHMP) Summary of Opinion

RRA	Required documentation
	• Summaries of other meetings with the MHRA (including presubmission advice, where relevant) • Approval letter • Post marketing review(s) (e.g., Periodic Safety Update Reports)
SwissMedic, Switzerland	• Comprehensive details of all studies submitted and assessed • All assessment report(s) • Questions from the regulator to the Market Authorisation Holder (and answers) • Summaries of meetings with SwissMedic (including presubmission advice, where relevant) • Approval letter • Post marketing reviews
United States Food and Drug Administration (US FDA)	• Comprehensive details of all studies submitted and assessed • Medical review(s) • Chemistry review(s) • Pharmacology review(s) • Statistical review(s) • Clinical pharmacology biopharmaceutics review(s) • Risk assessment and risk mitigation review(s) • Administrative document(s) and correspondence • Cross discipline team leader review • Office Director memo • Summaries of meetings with the US FDA (including presubmission advice, where relevant) • Summary review • Complete response letter

RRA	Required documentation
	• Approval letter • Post marketing reviews

Additional documentation requirements for the various types of applications may be stipulated in other sections of this guideline or other guidelines. Additional documentation requirements for WHO PQ products are detailed in SAHPRA's process for the WHO Collaborative Registration Procedure.

Additional documentation requirements for reliance-based review of variations applications are detailed in SAHPRA's Variations Addendum for Orthodox Medicines.

Please note that public assessment reports will not be accepted.

9. GOOD MANUFACTURING PRACTICE (GMP INSPECTIONS)

Good Manufacturing Practice (GMP) describes a set of principles and procedures that, when followed, ensure that medicines and related substances are of high quality, safety, and efficacy. SAHPRA is a participating authority of the Pharmaceutical Inspection Cooperation Scheme (jointly known as PIC/S). PIC/S aims to develop international standards between countries and pharmaceutical inspection authorities, to provide harmonised and constructive co-operation in the field of GMP. PIC/S affiliation is subject to initial and periodic assessment of the participating authority to ensure that it has equivalent legislation, regulatory and enforcement procedures, and inspection capacity. Besides employing a reliance approach to PIC/S affiliated authorities, SAHPRA Inspectorate also applies reliance to WHO, AMA and ZAZIBONA inspections.

9.1 Principles of Good Manufacturing Practice Reliance

GMP agreements with competent international regulatory authorities support information sharing and other desirable objectives for international regulatory collaboration. These agreements do not permit automatic acceptance of the decisions of the other party but may be used to enhance regulatory oversight and significantly reduce regulatory burden without diminution of compliance.

Manufacturers of medicines supplied in the South African market must demonstrate compliance with the relevant code of GMP. This is usually, but not always, done through an on-site inspection and with acceptable documentary GMP evidence. Hence, SAHPRA will not accept desk-top or virtual inspection reports or GMP certificate issued by the regulatory authority following a desk-top or virtual inspection.

GMP approval guidance for sites involved in the manufacture of products can be found below. **Please note** that adherence to these requirements does not guarantee a site will be deemed GMP compliant by SAHPRA.

SAHPRA reserves the right to request additional documentation, schedule an inspection, or reject any sites regardless of adherence to the below requirements.

The following requirements are applicable to finished products where the applicant has submitted GMP certificates or reports from recognized authorities for SAHPRA to implement reliance

- The site has been approved by a recognised regulator,
- The site was approved by the recognized regulator within the previous three (3) years. This date is calculated from the date of the inspection.
- The dosage form of the product within the application is within the same dosage form grouping as the dosage form approved by the recognised regulator,
- The product type applied for is the same as the product type approved by the recognised regulator, and
- The activities applied for by the applicant are the same activities that have been approved by the recognised regulator.
- The manufacturing site addresses the name or number of the manufacturing building or block or workshop should be clearly specified. This information should be the same as the information recorded in the application form.
- The validity and acceptability of the GMP certificate e.g. all pages included and no alterations.
- For Biological products, the GMP certificates submitted should include all relevant manufacturing sites from upstream manufacturing sites to fill and finish manufacturing sites.
- GMP inspection reports and certificates should be written in English, SAHPRA will not accept any GMP certificate or inspection report written in foreign language.

NOTE: Refer to the latest GMP guideline (**SAHPGL-INSP-02**) for the recognised regulators, dosages, product types, and activity groupings.

The following requirements are applicable to active pharmaceutical ingredients (APIs) where the applicant has submitted GMP certificates or reports from recognized authorities for SAHPRA to implement reliance:

- Date of inspection should be within 3 years from the date of submission of the application. This date is calculated from the date of the inspection.
- Health Authority should be one of SAHPRA Reliance Authorities for applications submitted from 01 July 2024 onwards.
- SAHPRA will only accept API GMP certificates or reports from local Regulatory Authority (country of manufacturing site) for applications submitted before 01 July 2024
- Approved API or API intermediate should match the API or API intermediate listed on the application form and in module 3 of the dossier.
- The API molecule(s) and grade e.g. USP, EP, EU grade should match the grade listed in the dossier
- The validity and acceptability of the GMP certificate e.g. all pages included and no alterations.
- **The CEP certificate is acceptable instead of a GMP certificate or report.**
- The manufacturing site addresses the name or number of the manufacturing building or block, or workshop should be clearly specified on the GMP certificate or inspection report or CEP certificate. This information should be the same as the information recorded in the relevant sections of the dossier.

GMP inspection reports and certificates should be written in English, SAHPRA will not accept any GMP certificate or inspection report written in foreign language. **FOR APPLICATIONS WITHOUT RELEVANT GMP CERTIFICATES, INSPECTION REPORTS or CEP CERTIFICATES FROM SAHPRA RELIANCE AUTHORITY – the following requirement applies:**

- The applicant should submit a request for a GMP inspection to SAHPRA to the following email address: inspectorate@sahpra.org.za with subject line: **Request for GMP inspection.**
- The applicant should also propose an inspection date when submitting the request for inspection for the manufacturing site(s).
- Proof of submission should be included in the dossier.
- Where a request for a GMP inspection has been submitted, SAHPRA will schedule an inspection for the relevant manufacturing site, and the outcome of the inspection will determine the status of the

application for GMP requirements.

- Applications without any relevant GMP certificates, inspection reports or CEP certificate or proof of request for GMP inspection will be evaluated as reliance applications and subsequently rejected if they do not meet the reliance requirements as outlined above.

10. CLINICAL TRIAL APPLICATION

Clinical Trial data is crucial in supporting the safety and efficacy of the product intended for registration. During the review process, the Authority considers information regarding the review status of the clinical trial with other Regulatory Authorities, as requested in the application form. As most of the clinical trials are multi-centre trials, the Authority will further take into consideration proper monitoring of trial and local conditions or prevalence of disease within the context of South Africa.

11. PHARMACOVIGILANCE

Vigilance is important for ensuring that health products available on the South African markets are safe, effective and of acceptable quality and performance throughout the life cycle of the product. To ensure that the Authority fulfils its mandate of monitoring the benefit-risk profile of the health products, the Authority will consider the safety information communicated or actions taken by other RRAs. The Authority considers and gives significant weight to assessments performed by RRAs in reaching its own regulatory decision. Furthermore, SAHPRA is an independent authority and is, therefore, responsible and accountable for the decisions taken, even when it relies on the decisions and information from other Regulatory Authorities.

12. LOT RELEASE

The requirement for routine independent lot release testing will be based on a risk assessment to determine whether SAHPRA can apply reliance. The risk assessment considers the post-marketing experience related to the safety and quality of the product. Reliance may be considered subject to the availability of a Lot Release Certificate issued by a releasing NCL from a WLA or SAHPRA RRA (see Section 4.1 above). Reliance can also be considered on an NRA designated ML3 provided that the product is WHO prequalified (see Section 4.1 above).

13. REFERENCES

The following related documents are referenced:

- 13.1 **Sameness declaration for Reliance-based Evaluation Models:** Appendix 2 to SAHPGL-PEM-02

Quality and Bioequivalence Guideline

13.2 SAHPRA GMP Guideline SAHPGL-INSP-02

14. VALIDITY

This guideline is valid for five (5) years from the effective date of revision and replaces the old Reliance Guideline (i.e. revision 6). It will be reviewed within this timeframe or as and when required.

APPROVED